Medical College of Virginia
Department of Medicine

PULMONARY LABORATORY REPORT

Name Swingle, Harold W. Date 9-12-72 Age 46 PFL#

Address 1404 Ford Ave. Race W. Sex M.

Mechanicsville, Va. Chart# out-pt. Location OP

Diagnosis Referred by: Dr. Baker

I. ARTERIAL BLOOD

	Normal Values*	#1	#2	#3	#4
Oxygen Tension (P_{O2})	90 - 100 mm Hg	72	62	625	
Hemoglobin Oxygen Saturation	96 - 98%	94	90	100	
CO_2 Tension (P_{CO2})	36 - 44 mm Hg	32	36	32	
pH	7.36 - 7.44	7.43	7.37	7.41	
Plasma Bicarbonate **	24 - 27 meq/L	20.5	20	21.c ~~20.5~~	
Hematocrit					
Respiratory Rate		20	28	20	
Time Sample Collected		1452	1452	1515	

Conditions at time of sampling:

#1 Ambient Air

#2 AA ex. 50 steps

#3 100% 02 for 15 min.

#4

* For Young Adults

** Calculated from P_{CO2} & pH

Comment:

Arterial blood gases show mild hypoxemia at rest with mild hyperventilati and a mild compensated respiratory alkalosis, After standard exercise th is a significant drop in 02 tension with slight decrease in ventilation a unmasking a mild compensated metabolic acidosis. After 100% 02, oxygen tension rose to normal value for this laboratory. The above changes are consistent with a diffusion abnormality rathet than right to left shunt.

George M. Bell, M.D.

2025039029

Source: https://www.industrydocuments.ucsf.edu/docs/nlgb0106

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PULMONARY LABORATORY REPORT

Name Swingle, Harold Willis Date 9-12-72 Age 46 PFL#

Address Ht. 63" cm. Wt. lbs. Race

Chart # Sex

Diagnosis Referred by Baker

Location OP

II. SPIROMETRY

Test	Units	Normal Values	Before Broncho-dilator	After Broncho-dilator
Vital Capacity	liters	3.48	2.79	2.93
	% pred.	100±15	80	84
One-second Forced (FEV_1)	liters		1.17	1.13
Expiratory Volume	%actual	80	44	39
Three-second Forced (FEV_3)	liters		1.80	1.76
Expiratory Volume	%actual		68	60
Total Fast V. C.	liters		2.66	2.93
Total time of expiration	seconds		9 sec.	13 sec.
Maximal Voluntary Ventilation	liters			
(MVV)	% pred.			
Maximal Mid-expiratory	L/sec.	4.37 ± 1.10	0.36	0.27
Flow Rate (MMEF)	% pred.			

Comment:

Spirametric examination shows vital capacity at lower limits of normal and severe reduction in expiratory flow rates with prolongation of expira time. After administration of a bronchodilator there was mild improvemen in vital capacity but no improvement in expiratory flow.

Conclusion: Spirametric examination shows severe obstructive pulmonary disease with no significant restriction as one might expect from the blood gas values.

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